

Anti-Social Services

Child Protective Services (CPS) is the generic name for the state social services agencies entrusted with protecting child welfare, including the investigation and prosecution of cases of suspected child abuse. Obviously, therefore, CPS functions as an important auxiliary in the family court system. Unfortunately, it also helps to re-constitute as an ongoing practical accomplishment, the madness of the family courts. CPS agencies are charged with the responsibility of assisting families in crisis and protecting abused children. When, in fact, they do just the opposite, it seems to us that they deserve to be called “anti-social services.”

When does that occur? CPS agencies can display quite a few kinds of anti-social behavior when it comes to protective mothers. First, they can bring bogus petitions against the mother, charging her with “brainwashing” her child into believing they have been sexually abused; then the agencies can come to court and argue so vigorously against the mother that she will lose custody to the ex-husband, even when substantial evidence exists that he sexually abused the child. Second, when child visitation is in CPS’s control, the agency can arrange for supervised visitation between the mother and child that is so restrictive and confining it becomes almost meaningless. A third kind of anti-social behavior is search and seizure: CPS agencies can claim for themselves the right to make unannounced visits to the home of a protective mother, supposedly to “observe” but often to comb her personal belongings for such damning “evidence” as books dealing with child sexual abuse—which

can then be cited in court as “proof” that the mother is inventing the abuse charges.

Whatever tack an agency may take, the depressingly common bottom line is that when CPS is out to get the protective mother—as happens all too often—the mother’s experience of the legal process is apt to be Kafkaesque. She may be accused in formal legal papers by people she has never met, charged with things that are not against the law, and yet find herself fighting to keep her rights to her children—or even her right to visit them—against not only a hostile ex-husband but a powerful government agency. She may even find that the agency is more committed to punishing her than to determining the children’s welfare. Many protective mothers have found, to their dismay, that when CPS agencies inject themselves into a case involving allegations of child sexual abuse, the outcome can be worse for the mother than if no agency had been involved in the first place. In fact, CPS agencies’ anti-social behavior can have effects that last long beyond the end of a trial. Protective mothers in the grip of hostile CPS agencies can be so affected by the agencies’ treatment that their life choices—a future marriage, decisions whether to have more children, where to live—come to revolve around their perceived status as “neglectful” mothers, regardless of how they made decisions before being involved with an agency.

BOGUS PETITIONS

CPS agencies are entrusted by state governments with the responsibility of investigating child abuse reports. If the agency believes a report is well founded, it files a formal petition with a family court, supported by a sworn affidavit from an agency employee, asking the court to declare the child in question “abused” or “neglected” so that the court can take appropriate steps to assure the child’s safety. This, at least, is how the system is supposed to operate.

But our research of family court cases shows that CPS agencies often abuse the process, filing petitions simply to inject themselves into divorce litigation whether or not they believe a child has actually been abused. In many protective mother cases, they file their petitions to gain control of the accuser, who thus suddenly finds herself at the mercy of the very agency that is supposed to exist for her children's benefit. Examples of bogus petitions appear later in this chapter, but there are certain common patterns that deserve mention here. First, petitions against protective mothers may be based on allegations that do not constitute child abuse or neglect as defined by the law—they simply reflect CPS's desire to take control over a woman and her children. Allegations that a mother "conveyed false information" to doctors, that she made an abuse report that "was not totally true," or that she failed to "insulate" a child from the legal proceedings are some of the claims by means of which CPS has removed children from their mothers.

Second, CPS petitions may contain statements that are simply false. CPS agencies are permitted by law to include allegations in their removal petitions that are based on suspicions, "tips," or other less than reliable claims. These claims are rarely questioned by the family court judges to whom CPS turns for orders removing children from their mothers' homes. The mother—who might be able to disprove the charges—need not be informed in advance if the agency claims an "emergency," which it often does. By the time the mother even knows what she is being accused of, a CPS caseworker can be at her door with a signed court order calling for her children's removal—and a police officer to see to it that the order is enforced. Even when the allegations are transparently inaccurate, they are usually good enough for family court. In one case, for example, a mother was charged with "child neglect" after the child accused her divorced father of sexually abusing her during a visit at her mother's home while the mother was away and the child's grandmother was caring for her. The mother was charged on one of two alternate grounds: either she had failed to protect the child from abuse,

or she had made a false abuse report. Both claims were literally impossible, the first because CPS's own case notes showed that the mother had been absent at the time (the child had been left temporarily in her grandmother's care), the second because CPS's notes showed that it was the grandmother, not the mother, who had actually phoned in the abuse report. But this did not prevent the child's removal.

In fact, in many cases, CPS removal petitions are sworn to by someone other than the caseworkers who investigate the facts—almost as if CPS officials want to ensure that those who sign can always claim ignorance of the truth if a rare family court judge actually holds them to the requirements of perjury statutes. Third, CPS agencies can fall back on the “get the kids first, explain later” approach. In these cases, CPS claims an “emergency” that justifies an immediate removal without the usual legal procedures. They may not even file a petition with a court. Once the children are removed from their home, CPS caseworkers grill the frightened, disoriented children for some evidence of maltreatment. When did they last see a doctor? Do they ever play in the street? Does Mommy ever hit them? Shout at them? Is someone watching them at literally all times? Are they dirty? If CPS can make it appear, from such interviews, that there is any possibility of sustaining a child abuse or neglect charge, the illegality of the original removal will generally be overlooked.

NONVISIT VISITS

CPS generally performs the supervision of visits when such supervision is required by a court order, after a petition alleging child abuse is filed or after a child has been determined to have been abused or neglected. As we have mentioned, protective parents are often the targets of such petitions. When children are removed from the custody of protective mothers, the *mothers* find their visits under the same CPS supervision

usually enforced against alleged offenders. Supervision of visits is meant only for the protection of the children. But supervision can mean much more than that to a hypervigilant CPS agency. It gives the agency unfettered discretion to make visits torture for parents and children.

Restrictions imposed by CPS in many of the cases we have studied include the following: parents are not allowed to whisper or speak softly to the children; parents are not allowed to give or receive written notes; parents are not allowed to hold their children in their lap; parents are not allowed to touch the children at all; parents are barred from bringing plastic utensils to eat cake; parents are not allowed to make any reference to the caseworkers; parents are not allowed to say anything about the case, even if the children ask about it; and parents are not allowed to correct their children if they repeat false statements about their parents they have heard from caseworkers.

In addition, caseworkers often intimidate parents by busily writing notes during visits—notes they routinely refuse to share with the visiting parent but will share freely with a hostile law guardian or judge. These notes are often written with a predetermined agenda in mind and detail the “inappropriate” behavior of the visiting parent, such as whispering, “excessive” hugging—even the expression of “too much” love (which caseworkers can say is an application of emotional pressure).

SEARCH AND SEIZURE

CPS can claim even more extraordinary powers over a parent’s life. Besides supervising visits, CPS has the power to inspect a parent’s home (without obtaining a search warrant). This means that caseworkers may arrive at any time (announced or otherwise), inspect anything, and then make reports that will later appear in court. They can also subject a mother to endless mental health examinations and have broad license

to invade her privacy by demanding releases for personal information such as medical reports, therapist's reports, and the like.

CPS home inspections may concentrate on mundane details such as the tidiness of a home (untidiness on the part of a protective mother may be treated as if it suggested child neglect) or on more specialized issues such as the presence of books describing sexual abuse and sexual abuse allegations (which may be presented as evidence that the mother is manufacturing allegations or is obsessed with concerns about abuse). However CPS goes about creating an unflattering record of a protective mother's life, habits, and character, the targets frequently find that the process is very hard to stop—it has a life and logic all its own. Once a protective mother becomes “known” to CPS, she has a “record” within the agency that is more than just a paperwork trail. Such records are “running records”—oral instruments that function as selective dossiers (which are based more on the “line” or “rumors” trailing a person than on the substantive facts)—that can be invoked at key points in a case. Such running records fill in where the facts leave off; consequently, they may be used to paint a picture of “neglect” or “abuse” even when the facts do not justify it.

UNCHECKED POWER, SECRECY, AND CONTROL

CPS malfunction stems from its very nature. Of particular concern are its wide-ranging powers and the secrecy within which it operates.

These factors—already touched on—contribute to a third problem, which is an attitude among CPS officials that they alone should control the decisions and behavior of the families they deal with. In 1992, Lucy Billings, former director for Special Litigation and Training at Bronx Legal Services, a state-funded program that provides legal representation to indigent clients (and now a judge in the Civil Court of the City of New York), testified before the US House Subcommittee on Select Education

of the Committee on Education and Labor. In her testimony, she described how CPS agencies misguidedly make unilateral decisions for a child by excluding the mother from that process: [A]gencies mistakenly act as if they have been given total control over the daily life and future of the child and, without parental involvement or consent, arrange for evaluations, services, and the sharing of information. It is common for a contracting agency [a foster care agency] to perform psychiatric assessments of children in foster care without prior discussion with the parent, her consent, or fully briefing her as to the outcome. Thus, in addition to not involving the parent in decisions and not obtaining consent, agencies presume it is their right to determine what to share with the parent. Psychiatric examinations of children performed by the agency employees are not discussed with the parent in advance nor after completion. These reports, however, contain information regarding her children that she has every right to know.¹

Billings also pointed out how CPS misapplies the *confidentiality* (read: secrecy) rules:

When family members are trying to address the family matters to which the Child Welfare Administration records on the family pertain, the CWA employees, without legal basis, resist disclosure. In contrast, CWA employees freely divulge such information to third parties who have no business or need to know all that is disclosed.²

In other words, “confidentiality” works selectively: mothers are barred from information about their own children that is freely shared with strangers—even when the strangers have no need to know it.

Furthermore, Billings identified a problem that is almost unique to the family court system: mothers are denied information by *everyone* concerned in a family court case; it is not unusual that even a mother’s own attorney, on instructions of the court, will withhold, for example, psychological reports on the mother written by court-appointed experts.

Similarly, mothers in family court cases have frequently complained to us about conferences between a judge and the lawyers, during which the mother herself is kept waiting outside while important decisions about custody and visitation are made in her absence. In no other specialty of law is a client likely to be treated this way.

PLAYING JUDGE

CPS's casual assumption of unchecked power has several practical results. One is the ease with which CPS "plays judge." That is, CPS agencies whose involvement in a case is governed by a court order will often assume powers beyond those actually in the order. In some cases, CPS has imposed restrictions on mothers not only in excess of a court order but even in violation of the statutory guidelines that regulate social service agencies—as when CPS prevents a parent from writing uncensored letters to her children, or arbitrarily denies parent-child visits.³ For instance, in the late 1980s, CPS prohibited an Oregon mother from writing letters to her nine-year-old daughter while the girl was in foster care, even though there was no order from a court to that effect.

Visit supervision by CPS calls forth some of the greatest agency creativity in imposing harsh and excessive conditions. We have already mentioned the "no whispering" rule imposed on many protective mothers during visits with their children. In one case, CPS barred a mother from holding her young children in her lap, for fear that she might bend over and whisper into a child's ear. Such orders are seldom stated by a judge or even mentioned in court—but CPS can generally count on family court judges to rubber-stamp their rules whenever called upon. When a Long Island, New York, mother complained at a court hearing in early 2000 about CPS's "no whispering" rule, the judge issued a written order on the spot prohibiting whispering—although there had been no such prohibition to begin with.

CPS prohibitions can border on the bizarre. One New York protective mother, for example, in a case from the mid-1980s, was barred from taking her six-year-old daughter into a bathroom stall without a caseworker present. This was not a case involving allegations of physical abuse against the mother; instead, the caseworkers claimed that if the mother was alone with the child in the bathroom stall she might use the “private” time to “brainwash” her child. In the same case, CPS imposed strange restrictions even on the child’s maternal grandmother, who was denied visits with the girl (no reason was ever given on the record) and was sternly instructed not even to touch her. One day, when the grandmother had accompanied the child’s mother to the family court building where supervised visitation was about to take place, she saw the child enter the room with a caseworker and leaned forward, smiling. The bailiff who monitored the visitation room rushed over and angrily told the grandmother, “If you move forward one more inch you’ll be arrested, ma’am!”

Supervised visitation not only provides a setting for draconian restrictions—it also gives CPS a golden opportunity to manufacture evidence that serves to prejudice the mother in court. Her word is rarely of much value against caseworkers’ reports.

For example, in a 2001 New Jersey case, a mother’s visits with her children were supervised by CPS caseworkers who were openly hostile to the mother. At the end of one supervised visit, the caseworker could not find the children’s coats. Immediately, in front of the children, she confronted the mother and accused her of taking the coats. The mother denied having taken them, but the caseworker angrily persisted in her accusations until the mother actually ran down the stairs to escape her. One of the boys started to follow his mother and was pulled back by the caseworker. The caseworker reported this to the family court judge—blaming it all on the mother, although it was never proved that she had anything to do with the missing coats. The judge obligingly wrote in an official decision that the mother “took the boys’ coats and wouldn’t

return them” during a visit. He even claimed the mother had “forced” the caseworker “to chase after her with the boys in tow.” This went far beyond the evidence. But it is rare for family court judges to challenge the CPS version of such events. Indeed, in this case, this incident was specifically cited by the judge in his determination that the mother had “emotionally neglected” the children.⁴

It is common for caseworkers and their supervisors to hover over a mother with legal-size notepads, obsessively taking down everything she says to her child (and CPS employees) or what the caseworker thinks she in fact “meant” to say. Some social service agencies have hidden tape recorders in the visitation room so that not a word of what the mother says will be missed. But this attention is not meant to build an objective record, as one New Jersey mother discovered when her daughter, during a supervised visit, made comments the mother thought suggested manipulation by the father. She asked the CPS caseworker who was taking notes to make sure those comments were written down. The caseworker told her that she had no say in what was recorded and that if she made any other comments about the note-taking she would not be permitted visits at all.⁵

One almost obsessive priority CPS usually brings to supervision of protective mothers is the fear that the mothers will abscond with their children. It takes very little to trigger this fear. For example, the New York protective mother who was supervised even in bathroom stalls was allowed once—exactly once—to take her six-year-old daughter to her home for a visit, a setting she and the child both preferred to the bare office at the agency building where visits had been conducted for more than a year. When she returned her child to the agency after the visit, the caseworker promptly said to her supervisor, “Well, we don’t have to get that arrest warrant for her after all.” The mother never knew exactly what she had done to move CPS so close to seeking her arrest, but this was the last “normal” visit the mother was allowed to have with her

daughter. At CPS's insistence, the family court ordered that the mother never again be allowed to take her child home, even for a visit.

One particularly excruciating aspect of living in the CPS fishbowl is that its complaints about a mother it dislikes can change whenever a new charge better suits CPS's convenience. For instance, in one case, CPS repeatedly accused the protective mother of making hostile comments about the father during her supervised visits with the child. CPS used this accusation to try to persuade the judge to terminate the mother's visits altogether. To disprove the allegation, the mother began bringing her own mini-cassette recorder to the visits, unobtrusively taping the visits by keeping the recorder inside her jacket. When CPS found out about this, it stopped accusing the mother of denigrating her ex-husband—but immediately condemned her for invading the child's privacy by taping her.

CPS also flexes extralegal muscles during home visits to inspect the "living conditions" of protective mothers. A New York mother with two young daughters, who alleged being subjected to repeated sexual abuse during visits with the father, was herself subjected to weekly "surprise" visits by CPS caseworkers. On one occasion the caseworker found books on sexual abuse in her bedroom drawers. Although there was certainly nothing illegal (or even surprising) about this, the caseworker confronted the mother on the spot with the suspicion that she was "planting" sexual content in the minds of her daughters. This mother eventually lost all confidence in the system and fled, along with her girls. A Brooklyn, New York, mother suffered a different sort of intrusion. Apparently because the mother believed that her daughter had been sexually abused (and was therefore, by definition, a flight threat), a caseworker was assigned to follow the mother whenever she traveled from one borough to another. At other times, the same caseworker would park her black sports car outside the mother's home for hours, sometimes for half a day, while closely observing who was going into and out of the house.⁶ The mother had not been accused of neglect and had no criminal record,

but she found herself under the sort of surveillance normally reserved for dangerous criminals. Some mothers get a taste of how CPS can “play judge” even before their cases get to court.

For instance, in 2001, a West Virginia mother’s three-year-old child came back from a visit with his father exclaiming, “My daddy hurt my pee pee; put medicine on it!” The mother contacted the Department of Human Services to seek the filing of an abuse petition against the father. Instead, according to the mother, once CPS officials met with the father (and his lawyer) the CPS caseworker assigned to the case solemnly told her that “people can lose their children for making up stories” and added, “it would be best to just drop the whole issue” as “it would be a ‘shame’ if she were found to be the problem.” The mother told the authors she definitely heard this as a warning, “a very clear message to shut up and not make any more waves.”

PLAYING GOD

CPS agencies can do more than play judge. As Lucy Billings (now New York City Civil Court Judge Billings) commented in testimony quoted above, CPS can take upon itself the authority to reshape a protective mother’s family, to make all her decisions, even in some cases to make judgments that judges themselves cannot make. In other words, they play God.

In 1993, Dr. Marta Lundy, then assistant professor of social work at the University of Illinois’ Jane Addams College of Social Work, was surprised to receive a formal letter summoning her to a meeting with the commissioner of the Illinois Department of Children and Family Services. She soon found herself the object of high-pressure treatment by CPS. Her crime? She had made a report of child sexual abuse to the department.

Dr. Lundy had been conducting research under a federal grant into the hidden costs (e.g., financial, vocational, and relational) to protective parents and their families that resulted from their reporting of sexual abuse to authorities.⁷ During an interview conducted by one of Dr. Lundy's research assistants as part of the study, a child disclosed that she had just been sexually assaulted by her father.⁸ Since Dr. Lundy, a social worker, was required to report incidents of sexual abuse under child abuse reporting laws, she phoned the child abuse hotline and reported what the child had said. That was when she received her summons.

When she appeared before the CPS commissioner, she was sternly warned that she was not allowed to report sexual abuse disclosed by that child, because "the local family court judge forbade any CPS investigations in this case."

In other words, Dr. Lundy was told that the mandatory reporting laws were in practice overridden by "higher authorities"—namely, a family court judge and CPS—when they considered it undesirable to hear any more about a particular allegation of child sexual abuse. What was more, CPS had thought so much of its power to override reporting laws that it had issued a written summons to the social worker to ensure that the reporting laws would not be followed in that particular case. Dr. Lundy has told us she found it "utterly shocking" that a social worker could be "reproached" by the commissioner himself for "fulfilling her responsibilities in the first place" to "report child sexual abuse."⁹

This event prompted Dr. Lundy and her co-investigator, Dr. Carol Rippey Massat, to redefine the extent of their study so as to include the phenomenon that is the subject of this book—that is, the "penalization" of mothers, and others, for reporting sexual abuse.¹⁰

Less than a year later, after Dr. Lundy and Dr. Massat had seen several cases in which CPS penalized mothers for making good-faith reports of sexual abuse, Dr. Massat took the extraordinary step of writing to George Burgasser, then acting chief of the Child Exploitation an-

dObscenity Section of the US Department of Justice, to apprise him of this disturbing pattern: “The findings of a preliminary study funded by NCCAN showed a consistent pattern of punitive responses [to the non-offending parent] to the reporting of child maltreatment . . . a pattern of obstruction of investigations . . . in apparent contradiction of the legal requirements of CAPTA.”¹¹

Dr. Massat told one of us her personal reason for writing to the US Department of Justice. Just as a researcher may be moved to take action during a study when it reveals public health dangers, she said, what she had seen of the penalization of protective mothers warranted some action, such as a Department of Justice investigation. (The Department of Justice did not respond.)

Another story reveals a similar pattern of overreaching by CPS. In January 1996, the *New York Post* reported that New York City’s Child Welfare Administration (CWA) had suspended Rosalie Harman, a senior-level supervisor, for purportedly “violating the agency’s press policy.”¹² But according to the *Post*, “agency sources said Harmon [sic] has been a target of retribution by CWA officials since 1993, when she testified without permission before a state Senate panel about a bitter custody battle” (p. 4).

That “bitter custody battle” involved a child who was abruptly removed from her mother’s custody and placed in foster care for nearly a year and a half because the mother had allegedly made a “false” report of sexual abuse. (In fact, mental health professionals confirmed the girl’s credible disclosure of sexual abuse, which had originally been reported by an eyewitness—not the mother.) The case attracted public and legislative attention when the then eight-year-old girl nearly died of anorexia nervosa after being transferred to her father’s custody. Harman, who heard about the case after a fresh report concerning the child’s alarming emaciation, obtained CWA’s file on the case in an effort to learn how the child’s condition had deteriorated to such an extent. What she found in the file prompted her to write, in a sworn affidavit:

The Brooklyn field office worker pressed for a physical examination of the child ... [and] the father maintained he did not want to be investigated for child sex abuse. For some inexplicable reason the case was closed.... The sex abuse protocol therefore was incomplete.¹³

Senator David A. Paterson, who chaired a New York Senate hearing that investigated the agency's misconduct in this case, told the Post that at the time Harman testified before his committee about her agency's mishandling of this case, she expressed a fear of retaliation. Thinking back on it, Paterson said, "I should not have allowed her to testify. I have always wrestled with my conscience about the peril I exposed this woman to within her agency" (p. 4). Paterson was one of several who believed that Harman was being punished because of her testimony—that is, because she had told the truth about a mishandled investigation of alleged child sex abuse in which the father was given preferential treatment and the child's welfare was jeopardized.

A week after Paterson shared his suspicions with the Post, he repeated them in a letter to Lee J. Radek, then bureau chief of the Public Integrity Section of the US Department of Justice:

A key whistle blower at my 1993 investigative hearing into the [name withheld] case made headlines last week when she was suspended without pay. The penalization of this whistle blower came as no surprise to me in that I had been apprised of the fact that she had been reproached [by her superior] in front of several of her colleagues for [making] inquiries into the [name withheld] case. I, too, as an elected state official have met only with the most severe form of resistance each time I've attempted to investigate malfeasance of agencies and judges over the handling of the [name withheld] matter. Quite honestly, I found the reaction of agency officials, judges and elected officials to be formidable and frightening. My concern is that the punishment of this whistle blower will have a chilling effect on other

potential informants unless there is a concerted effort made to crack open this case once and for all.¹⁴

One final way in which CPS agencies play God in protective mother cases is by assuming the power to redefine a case's facts and legal procedure in order to change its outcome. A District of Columbia case illustrates how CPS officials can do this, playing judge and God at the same time. The mother in this case lived in Maryland, where she was litigating a divorce from a man she believed had sexually abused her two boys, aged four and seven. The Maryland court heard "emergency" allegations from the law guardian that the mother represented "a very real risk of flight with the children," urging the court to restrict her to supervised visits with her children. The judge, however, refused.

But the law guardian was in simultaneous contact with a "higher" authority: the CPS agency of Washington, DC, where the father lived. Even as the Maryland court was hearing actual evidence, and deciding that no emergency existed, CPS caseworkers, based entirely on hearsay and guesswork (without even a pending DC case to give them jurisdiction), were already deciding that they knew better. Before the Maryland court had discounted the charges against the mother, a caseworker for the District of Columbia CPS had already written a memorandum stating that her agency wanted to "bring the case into DC Courts" in order "to have the visitations with the mother changed to supervised."¹⁵

To accomplish this result, CPS did not hesitate to rewrite the facts of the case that was proceeding in Maryland. The caseworker's memorandum contained several serious inaccuracies:

- The caseworker stated that the "children are continuing to be placed in the middle of the parents' battle." In fact, two separate evaluators had already *refused* to conclude that the mother was coaching or manipulating the children.¹⁶
- The caseworker wrote: "It is feared that the mother may kidnap the children and run or harm the children in some way." This is

precisely what was alleged in the Maryland court, but no evidence was ever presented to support the charge, and the only court that heard the charge rejected it.

- The caseworker claimed that an evaluator appointed by the Maryland court had said that the mother “has no rational thoughts at this time,” that she “is getting worst [sic],” that her “disorder is hard to treat,” and that she “fired 45 of her attorneys.” *Not one* of these statements appears in the evaluator’s extensive testimony.

On the very day the Maryland court signed an order refusing to find an emergency or to limit the mother’s custody or visitation, CPS in the District of Columbia filed its own petition—thus commencing the case all over again in a new jurisdiction—and seized the children, refusing the mother access to them. CPS restricted the mother to supervised visits of two hours per week and even cut off her telephone contact with the children. The father, meanwhile, was given custody and completely unrestricted access to them both, despite the abuse allegations against him.

The allegations against the father did not lack evidentiary support. A therapist documented that the children reported to her, spontaneously, that their father had kicked the elder boy in the crotch, tickled his private parts, withheld food from him, and threatened to kill him and his mother by setting fire to their house. She also reported that one of the elder boy’s teachers had observed him flinch when the teacher placed his hand near the boy’s head. The boy said he was afraid to visit his father.¹⁷ But CPS appeared quite ready to leave the facts behind. For example, its petition charging the mother with “neglect” claimed that the Maryland court had already given the father primary custody. That was untrue: the parents had shared equal time and custodial rights with the children.¹⁸ The petition also claimed that the children had “changed their story” about the alleged abuse by their father in January 2001 to be “consistent with [the mother’s] account of events”—a very suspicious-

sounding circumstance. Yet this claim, too, is not supported by the record.

As the months wore on, and the mother struggled in vain to have her children returned to her, CPS's claims against her—which, as we have seen, had never had much connection to reality—became increasingly bizarre. Eventually a DC police detective assigned to the case claimed in a sworn deposition that the mother posed a threat to the children because the “thought crossed my mind” that the mother “would enter into a murder suicide pack [sic] . . . with her and the children” if she lost custody to the father. As for how the detective came to believe this danger existed, she admitted she had not consulted a psychiatrist but said, “You know, you hear about it in the news all the time.”¹⁹ Perhaps she believed herself capable of reading the mother's mind—which is less surprising than it sounds, since CPS had already proved itself capable of reinventing facts and moving a case from one court system to another to achieve the results it wanted.

FALSE CPS RECORDS

We have seen (in Chapter 5) that law guardians can produce records that are selective, or even false, in order to serve the “contractual” purpose of persuading a family court judge to punish the mother. Anti-social service agencies can do the same thing. In fact, the unchecked power and secrecy that characterize CPS agencies make them a perfect breeding ground for false or misleading records. Facts are easily distorted in an environment isolated from scrutiny.

Suppose a CPS agency files a petition against a mother that contains provably false statements. In theory, this should be easy to correct. In practice, even patently false statements in a petition can have very serious consequences for the protective mother. For example, a false “tip” that a mother is mentally ill and dangerous may prompt a judge

to order psychological evaluations before anyone bothers to check the accuracy of the charge. The evaluators, knowing from the petition what CPS wants to find, can then determine that the mother has a “histrionic” or “narcissistic” personality, or is overly “enmeshed” with her children, or suffers from one or another of the vague diagnoses favored in family courts. This, in turn, gives the judge a rationale for removing the children from the mother, even if it turns out (as it often does) that the original “tip” was completely baseless.

False records can become “true” in another way as well. Certain actions, such as child custody, can become self-justifying over time. That is, after a child has been removed from its mother and placed with the father, the child will gradually accept its new environment (even if originally they vehemently objected to it), so that after enough time with the father CPS can resist a return to the mother’s custody by claiming this would needlessly “uproot” the child. Similarly, when a young child is placed in foster care and is told repeatedly that the only way out of a stranger’s home is to live with the father, most children, after sufficient time, will choose that alternative over foster care. In this way, false statements that result in children being removed from protective mothers gradually become “factoids.” The mother may prove them false and still never recover her improperly removed children. To make matters worse, there is really nothing to stop caseworkers from making false statements in the first place, whether in case note files or in sworn documents submitted to the court. This is because caseworkers generally enjoy legal immunity from lawsuits, even if their inaccurate statements in agency records or court documents wrongly cause a child to be removed from an innocent mother. We explore this issue—and possible reforms in the law—in greater detail in Part III, Chapter 11.

SPURIOUS NEGLECT PETITIONS

We have already seen that the formal complaints, or petitions, by which CPS agencies seek the removal of children from mothers may be more factitious than real. It is appropriate at this point to examine this phenomenon more closely.

The laws governing family courts throughout the country, although they differ in some details from state to state, provide a generally consistent definition of child neglect. New York law defines a neglected child as one “whose physical, mental or emotional condition has been impaired or is in imminent danger of becoming impaired as a result of the failure of his parent or other person legally responsible for his care to exercise a minimum degree of care.”²⁰ In Colorado, a parent is guilty of neglect if they have “abandoned the child or has subjected him or her to mistreatment or abuse or ... has suffered or allowed another to mistreat or abuse the child without taking lawful means to stop such mistreatment or abuse and prevent it from recurring.”²¹ Similarly, California law defines child neglect as “the negligent treatment or the maltreatment of a child by a person responsible for the child’s welfare under circumstances indicating harm or threatened harm to the child’s health or welfare.”²² It is noteworthy that none of these definitions includes the making of unfounded sex abuse allegations as a kind of child neglect. Indeed, even to the extent that these definitions embrace psychological harm to a child, they certainly seem to require much more than a showing that a mother suspects abuse by the father. The District of Columbia’s definition of “psychological abuse” is typical: it requires a showing that a mother suffers from an actual, definable mental illness and, in addition, proof that this mental condition poses a real threat to the well-being of the child.²³ In all the statutes cited here some actual danger to the child’s health or welfare is required in order to reach a finding of “child neglect.” Statutes authorizing a family court, at the urging of CPS, to remove children from their homes *before* the com-

pletion of a court proceeding—that is, before a parent has been found guilty of abuse or neglect—are even more strictly drawn. States must show that “removal is necessary to avoid imminent risk to the child’s life or health” (New York);²⁴ that “continuing the child’s place of residence or in the care and custody of the person responsible for the child’s care and custody would present a danger to that child’s life or health in the reasonably foreseeable future” (Colorado);²⁵ or that “[t]he child would be subjected to an imminent threat to life or health to the extent that severe or irremediable injury would be likely to result if the child were returned to or left in the custody of his parents . . . and . . . there are no alternatives less drastic than removal of the child from his home which could reasonably and adequately protect the child’s life or health pending a final hearing” (Virginia).²⁶ Again, these statutes represent the norm throughout the fifty states.

In practice, however, CPS applies extremely elastic definitions to these laws, making it possible for mothers to lose their children—even before a trial or hearing—for reasons such as the following: (1) spending too much time litigating; (2) failing to “insulate” children from the legal proceedings (even when the children are too young to understand the process, let alone to be brainwashed); (3) making an abuse report that is “not totally true”; and (4) changing therapists for her child without first obtaining permission from a family court.

It is evident that none of these claims fits the statutory definitions of neglect, let alone of an “emergency” justifying removal of children. Yet for each of these reasons CPS has sought—and obtained—the removal of young children from their mothers’ homes.

In fact, in some cases we have examined, CPS’s accusations were apparently not even believed by CPS. For example, a Brooklyn, New York, neglect petition charged the mother with having made a false report—but also charged her with having failed to protect her child from the reported abuse and charged the father with abuse as well! Obviously not even CPS could have believed all these allegations. And many of the

cases we studied included allegations against the father that CPS never bothered to investigate fully. Evidently, it suits CPS's ultimate agenda in many cases to allege the unfitness of *both* parents; after all, that makes removal to foster care that much easier to justify. And once children are in foster care, CPS can exert considerable control over them (control that can yield statements from the children in support of CPS's case) by making it clear that their release from foster care depends on their accepting the home CPS has chosen for them—as we will see in more detail later on.

Of course, CPS agencies would not generate spurious petitions if these were not generally accepted by family court judges. In practice, few judges demand that CPS agencies adhere to the legal standards set for them (and those who do, such as the New York family court judge described in Chapter 2, are likely to find themselves driven out of the system altogether). Not only is there widespread judicial tolerance of improperly founded petitions and removal orders; judges sometimes even press CPS to file a neglect petition on the flimsiest of grounds.²⁷ What follows are a few case histories involving spurious petitions for neglect and the ensuing orders removing a child or children from the mother's custody. They are typical of many similar cases. The "False Reporting" Mother In 1993, a young British-born Brooklyn, New York, Orthodox Jewish mother fled to a kosher battered women's shelter with her three-year-old son. She told workers there that her husband had been abusing her and the boy. (A health professional confirmed, anonymously, her suspicion that the father abused the child.)²⁸

In September 1995, the boy disclosed that he had been abused in his father's apartment—nicked on his penis by his father's apartment mate, then beaten savagely with a belt by his father—"because of the blood he had to clean up," according to the mother. Twice that month, she reported her suspicions to the CPS "hotline," which found enough support for her charges to list them as "indicated," child welfare jargon

for “substantiated.” Their reports noted a “small cut” under the boy’s penis and bruising on his buttocks.²⁹

The mother had particular reason to be concerned: a hospital report made in 1994 described the father as “chronic paranoid type,” displaying at times “acute mania, with paranoid, agitated, violent, irritable behavior.”³⁰

A Brooklyn court appointed a law guardian and an evaluator. But visits between father and son continued, and the mother, feeling that nothing had been done to protect her son, turned to the district attorney’s office for help. She met with a “counselor” for domestic violence and child abuse complaints, but still no progress seemed to result. Then, in May 1996, the case took a new turn when the mother brought the child to a hospital following further disclosures of sexual abuse. The hospital found no physical evidence of abuse this time, although the child reportedly said that he had been fondled on his genitals. But now the “counselor” at the District Attorney’s (DA) office, who had brushed aside the abuse reports, swung into action—against the mother. More than seven months after the mother’s first visit seeking help, she suddenly found herself summoned, with her son, to the counselor’s office for a second interview. The same day, the counselor finally phoned CPS with her own abuse report—seeking action against the mother. CPS was more than happy to oblige. The next day, its officials filed papers in Brooklyn Family Court accusing the mother of child neglect and seeking the boy’s “emergency” removal from the mother’s home. A family court judge issued the order, and the mad family court merry-go-round was off to a typical start.

No one who has read this far will be surprised to learn that the allegations used to obtain this “emergency” removal were wildly inaccurate. For example, CPS’s petition against the mother stated:

Since approximately March 1996, the respondent mother has made several *false reports* of child abuse/neglect to the State

Central Register alleging injuries to the subject child. On several occasions [the mother] has sought medical attention for the child and made *false accusations* of abuse or neglect to the medical staff.³¹

These sworn statements were untrue: as CPS must have known, the mother's charges had been at least partly substantiated. The reports were at most unproved; they were not "false."

CPS was not finished. The petition concluded: "The respondent mother has previously been diagnosed with mental illness and has exhibited homicidal tendencies."³²

These allegations were clearly intended to establish an "emergency" sufficient to justify the child's removal. But they were untrue: no mental health professional had ever diagnosed the mother with any mental illness, let alone with "homicidal tendencies"!³³ This was a critical inaccuracy.

Paradoxically, having charged the mother with making false reports, CPS rounded out its proof of an "emergency" by charging the father with, among other things, having "beat[en] the subject child with a belt" and standing silent while his roommate "cut the subject child on his penis, causing him to sustain a laceration." (In other words, the mother had falsely reported abuse, but the abuse had occurred.) This charge was intended only to justify the child's removal into foster care, however; once the boy was away from his mother, CPS forgot all about the charges against the father. As is usual in family courts, the falsity of the charges against the mother did not attract undue attention. The law guardian sided with CPS, telling a reporter that removal had been necessary because the mother "was making a series of accusations that . . . were unfounded." (As previously shown, New York law requires an "imminent" threat to the child's health or welfare to justify removal; how making unfounded accusations, even if this had actually occurred, could constitute such an imminent threat is anyone's guess.) Family court proceedings

dragged on for more than a year before the mother even got a chance to challenge CPS's claims—during which time she was allowed only two hours a week of supervised visitation with her son. This occurred despite the fact that the court-appointed evaluator's written report did not even describe the mother as dangerous.³⁴

The mother told a reporter that the law guardian and CPS repeatedly pressed her to agree to give the father unsupervised visits; if she did that, she said, they would agree to return the child to her custody. She held her ground for months, insisting that only supervision could adequately protect the child when in contact with the father. Eventually, however, she was forced to compromise. In exchange for continuing supervision of the father's visits, she agreed to leave the child's legal custody with the state, although she was given primary physical custody. This meant that the state had ultimate supervisory control; CPS had the power to "monitor" the mother for another eighteen months or longer.

During this time, the mother was subject to unannounced home visits by CPS at any time, and required to attend "therapy" sessions—always with counselors who worked for CPS or were recommended by the agency. She also had to report regularly to a CPS caseworker.

When CPS originally removed this mother's son from her home, the agency had claimed she had a mental illness and "homicidal tendencies." As we have seen, these claims could never have been taken seriously, and were in fact used only to obtain the removal and to keep the child under CPS control for over a year thereafter. Besides, the father quickly seemed to lose interest in visiting his son (and any visits he had were supervised), so there were rarely opportunities for additional abuse reports that could be claimed as evidence of the mother's "illness." But with the child officially in CPS's custody, even while living with his mother, CPS hit on a different set of allegations for the next step. CPS's constant interviews, "therapy" sessions, and unannounced visits gave the caseworkers ample opportunity to observe the mother's weaknesses

as a homemaker. The mother has described herself as suffering from Attention Deficit Disorder, and she often had difficulty completing several household tasks at once. Although no one had previously attacked her fitness as a mother—a CPS caseworker who visited the child at his mother’s home early in the case “observed child to be calm” and noted that he “appearance [sic] well”³⁵—caseworkers now began to describe the mother as “forgetful” and “sloppy.” They filled their case notes with references to her confusion at home, her unwashed dishes, the appointments she missed or was late for. Quickly, the mother achieved a new “running record”: as a mother so sloppy, distractible, and overburdened that she could not care for her child. This sounds almost comical, but it was not funny to the mother: she described her life as “constant pressure” and worried daily about the next CPS visit. She had doubts about her own competence. CPS, she was sure, would find something wrong with her house again. What would it be this time? What would happen to her? Could they take away her child again? What if they found something undone, something out of place? Needless to say, the mother’s anxiety did not improve her performance. Meanwhile, CPS caseworkers were not content with criticizing every aspect of the mother’s homemaking. They also steadily pressed her to place her child in foster care, even if just for a weekend. Between the constant criticism, the helpless sense of living in a fishbowl, and the repeated urgings to place her child in foster care, the mother began to give way. After months of urging from the CPS caseworker, she agreed to place her son in a foster home—only temporarily, she thought. CPS pressure did not stop there, however. Eventually the caseworkers persuaded this Orthodox mother to relinquish her parental rights altogether (and to have her child placed for adoption) lest she face a long, difficult court battle trying to prove her parental fitness. We have dwelt on the details of this case, which bear repeating—if only in summary—because they show how CPS intervention, through the family court process, can effectively reshape reality. The case began with some unverified and inaccurate entries in a CPS

file; it passed into false statements in a needless neglect petition; it ultimately became a war of attrition against a mother whose child had already lived more under the control of CPS than of his mother. After seeing the child welfare system take her son, hold him for over a year, place her under constant supervision, and threaten her with more attacks, the mother was unable to resist what had become, by then, a self-fulfilling prophecy. Even though she had fought vigorously to keep her son, she could not fight forever against an adversary that could change the rules whenever it chose.

The Mother with No “Energy and Time Left for Parenting”

In 1989, a New Mexico mother whose ex-boyfriend may have sexually abused their four-year-old daughter found herself charged by CPS with “neglect.” The reason? CPS alleged that the mother “invested so much energy and time in the legal issues” that she “may not be able to have energy and time left for parenting.”

How could a woman’s commitment to “legal issues” such as attempting to protect her child from an apparently abusive relationship constitute child neglect? Here is how CPS built a catch-22 for this particular mother:

During custody litigation between the couple (who were never married), the couple’s child reportedly showed signs of having been sexually abused. CPS records show that sex abuse “referrals” were made to the agency by two psychologists, one of whom was the child’s therapist. CPS also acknowledged that the girl “had intermittent vomiting and diarrhea following visits with the father,” as well as “sleeping problems, fear of the dark and weight loss.”³⁶ The mother’s attempts to persuade CPS to start official proceedings against the child’s father were unsuccessful; CPS claimed that the reports of abuse were not substantiated by physical evidence. However, CPS did offer the office of a “Court Clinic” for

visits in an attempt to neutralize the “conflict” that was clearly taking place between the parents.

The mother told us that the father would “erupt” whenever he saw her. An affidavit submitted by the child’s attorney confirms that the parents fought during visits. However, when the mother refused to bring her child for a scheduled visit, telling the clinic that she did not want to expose the child to any more of her father’s outbursts, she was arrested and jailed for contempt of court.

Not long afterward, the clinic decided that supervising these visits was too difficult. At that point, CPS immediately sought the child’s removal into foster care. CPS claimed this was necessary because “the Court Clinic does not have the staff to provide the necessary monitoring” and because “both parents have invested so much energy and time in the legal issues that they may not be able to have energy and time left for parenting.”³⁷ A family court judge promptly removed the child from her mother’s home.

In other words, CPS justified the removal of a four-year-old child from her mother because the mother attended to “the legal issues” and because of conflicts with the father that she says were not her fault and that, in any case, could have been contained by supervision! Just as amazingly, CPS’s petition did not allege any actual deficiency in the mother’s parenting abilities as a result of her attention to the case.³⁸ Family court placed the girl in the custody of the State of New Mexico Human Services Department. She stayed in a foster home for several months. And now CPS’s true goal emerged. Had its complaints been genuinely directed at “both” parents, as its papers claimed, it would never have recommended placing the child in her father’s custody. But that is what CPS did. Having achieved its goal of removing the girl from her mother, CPS never actually pursued the charge against the father; like the charge against the father in the previous case, this one was evidently only a means to get the child into foster care. Instead, according to the mother, CPS urged that the child be placed with the father,

who gained sole custody of the girl after a three-month interval of foster care. Ironically, the charge of “excessive litigation” then became a self-fulfilling prophecy, as the mother waged a two-year political campaign to regain her daughter, reaching out to support groups, news media, and politicians. CPS saw all this merely as “proof” of the mother’s unfitness, and eventually it recommended—successfully—that even her visits with her child be suspended.

Having lost her daughter entirely to CPS, the mother finally, painfully, began to rebuild her life. But the effects of the agency’s campaign against her remained for years. Increasingly suspicious, the mother hesitated to date; in fact, she told one of us that she feared having another child, worrying that CPS would take the new one away just as it had the first.

The “Brainwashing/Coaching” Mother

In 1986, a California case began hopefully for a protective mother when local CPS officials filed a petition against the father of a three-year-old girl, charging him with (among other things) “cutting” the “minor’s perineum [between the vagina and the rectum] with a knife.”³⁹ A caseworker testified in support of the charge. The child also made consistent reports of abuse and repeated them to a family court judge.⁴⁰ In response to all this, the judge ordered that the father’s visits be supervised by CPS, and he scheduled a hearing for a later date to determine whether visitation should continue to be supervised.

But before the next court date, according to the mother’s later testimony (submitted in writing to a congressional committee), the father applied for—and received—a transfer of his case to another court some thirty miles away. Immediately he began an aggressive campaign to take custody of the child from the mother.⁴¹

A new CPS worker was assigned to the case. According to the mother, the new worker *withheld* from the judge her colleague's petition accusing the father of child abuse—along with forty-seven pages of supporting documentation.

In August 1987, CPS, citing the recommendations of the court-appointed family therapist, filed a petition seeking the child's placement in foster care.⁴² Ignoring the existing record of abuse by the father, CPS claimed that removal was necessary because of "the custody dispute." Weirdly, CPS complained that the child—who had reported being abused by her father on several occasions—was "forced to play roles . . . in an attempt to please or appease her mother." The agency also complained that "the minor's mother behaves 'almost as if she is obsessed' with the belief that her daughter has been molested" and accused her of "coaching" the child to make reports of abuse.⁴³ Without further questions, the judge ordered the child placed in foster care for several months.

Thus, in classic family court fashion, the mother was judged more by her running record—the "party line" on her case, shared by auxiliary agents in collaboration with the judge—than by the facts. In this hostile environment, battling CPS and an "expert" who did not believe her, the mother struggled to protect her daughter from what appeared to be continuing abuse. The child returned from visits with burns, scratches, and bruises.⁴⁴ CPS turned a deaf ear, refusing to investigate at least eight reports of alleged child abuse made by doctors and the child's therapist.⁴⁵

When the child was released from foster care four months later, the father and mother were given joint physical custody, with legal custody retained by the state. The child continued to show symptoms of sexual abuse, including "genital warts, urinary tract infections, various vaginal infections, bruises on various places on her body and vaginal area [including] cuts, abrasions and lacerations."⁴⁶ But the reports were still ignored by CPS.

Finally, in February 1988, the child complained again of molestation by her father. Once again, the mother reported the disclosure to CPS, whose officials called her “irrational.” Desperate, the mother took the girl out of state, in defiance of a court order forbidding her to do so (since the child was legally in the custody of CPS). Freed from CPS’s restrictions concerning which doctors the mother could consult, the mother arranged for the child to be brought to a pediatric gynecologist in Oregon for testing. A colposcopic examination of the child’s genitalia revealed a dramatic fact: the girl had a sexually transmitted disease. In her report, the examining physician noted, “After what appeared to be condyloma lesions . . . on the perineum, a biopsy was taken in the right lower perineal area [showing] definite condyloma infection.” She summed up: “At this point, the conclusion has to be noted that the patient probably has contracted a known sexually transmitted infection, and therefore, the reasonable conclusion is that she has suffered sexual abuse.”⁴⁷

The pediatrician also told the mother that if these condyloma lesions had not been removed, the child would have run the risk of pelvic inflammatory disease and ensuing sterility. The mother later told one of us that when she received this medical report, she was certain that CPS would have no choice but to prosecute the father for abuse and that the child would soon be given back to the mother’s custody. Given the Oregon doctor’s conclusions, who could blame her for believing that?

But she was wrong. A member of the mother’s family contacted CPS and sent the agency a copy of this new medical report—showing that the young girl had a sexually transmitted disease—in the hope that the mother and child could return to California and that the child would be properly protected. The caseworker she spoke to, however, was unmoved. She insisted that the mother had “brainwashed” her child to believe she had been molested. Even physical evidence apparently meant nothing to an agency that had already made up its mind.

The family court judge had a federal warrant issued for the mother’s arrest. She remained in hiding for four years, until she and her daugh-

ter were caught in 1992. While she was in jail, awaiting trial on parental kidnapping charges, a friend testified before a Congressional subcommittee in her support. She told a panel of congresspersons that “during the continuing processing by CPS,” what should have been a straightforward abuse case “became convoluted and distorted.”⁴⁸ The mother was acquitted of the charge of parental abduction after she persuaded a jury that she had had no other choice under the circumstances but to abscond with the child in order to protect her from abuse. But incredibly, despite all the evidence that the child had been abused, the family court heeded CPS and eventually awarded custody to the father. On the day the court ruled in his favor, CPS “yanked” the child, according to her mother, “kicking and screaming,” out of school to take her to the father’s home. The child, then twelve years old, hardened by her experience in the “Underground Railroad,” fled within a few months to a “safe house” connected with the Railroad.

This did not mean a reunion with her mother, however. The “safe house” operator, to protect the child from discovery and the forcible return to her father that might ensue, advised the mother against visiting her, with the result that she did not see her daughter until the girl reached the age of eighteen and was at last free of the family court’s control.

By then, the years of futile court battles, oppression by CPS, and life as a fugitive had all taken their toll. The child had never received a formal education. The mother, not yet forty, was disillusioned and bitter. She told one of us: “I’ve suffered tremendously and I have very, very little expectation for society to ever reconcile with how cruelly the family courts treat sexually abused children.” Significantly, she perceived her own case not as an anomaly but as part of a larger picture. This fact deserves some consideration. What it illustrates, to an ethnomethodologist, is that the mother—through her own efforts to make practical sense out of the actions of the family court system—invoked the “documentary method of interpretation.” In so doing, she saw her own case

as “documenting” or “pointing to” an underlying social problem, namely, society’s failure to grapple with how the courts treat sexual abuse. And in defining that problem, the mother had particularly harsh words for the behavior of the CPS agency that had been charged with protecting her daughter and had instead torn her from the parent who had tried to protect her: “CPS just shook my beliefs in democracy and freedom in America.”

The Mother Who Communicated “False Information” to the Doctor

In 1991, a divorcing New York mother of two young girls, a registered nurse, was awarded sole custody of her two daughters, aged three and seven. Several months later, after a weekend visit of both girls with their father, the mother made a report to CPS, claiming a suspicion of sexual abuse. CPS took no action; the father then persuaded the family court to move the children to his custody, claiming the mother was unstable. About a year later, while the mother was litigating for the return of her daughters, the younger child complained to her of pelvic pain. She also had a high fever. The mother took her to a hospital, where doctors were baffled by the symptoms. The mother suggested the child might be suffering from “pelvic inflammatory disease” (PID), and explained that such inflammation could have resulted from sexual abuse. The mother went on to complain to the doctors that CPS had not told the family court all it knew about the medical evidence of her daughter’s alleged abuse.

Thanks to the information given by the mother, the doctors made a report to CPS that child sexual abuse had been alleged.

A few days later, the mother was stunned by the arrival of a formal petition accusing *her* of neglecting her daughter. She was particularly mystified by the specifics of the accusation, which were based entirely on her attempt to aid in her daughter’s diagnosis and her criticism of

the agency. Evidently CPS believed that this amounted to a form of child abuse:

Upon information and belief, on or about February 1, 1993 the Respondent [mother] caused false information to be given to representatives at Nyack Hospital resulting in the filing of a report of alleged child abuse or maltreatment [against the father] which alleged, inter alia, that the child has been sexually abused “regularly” by [the father]. Specifically, the report contained the false statement provided by the Respondent or her representative, that “child has a history of pelvic inflammatory disease;” and that the local Child Protective Services “withheld information from Family Court, given to her from [a child abuse expert], in which there was medical evidence that the child was sexually abused”⁴⁹

CPS also cited the mother’s efforts to publicize her children’s plight as further evidence of “child neglect”: “Upon information and belief, in or around January and February 1993, the Respondent distributed a ‘flyer’ in and around [the child’s school], and Nyack Hospital while [the child] was a patient . . . alleging, inter alia, that her daughter without specificity has been sexually abused by [the father]” (p. 3).

The filing of this neglect petition was enough to prevent family court from returning the girls to their mother’s custody. After all, how could a judge give children back to a mother who had been charged by a state agency with “neglect”? But the charges in the petition were baseless: the evidence indicates that the mother gave a full and accurate medical history. And how could her attempts to enlist support in her effort to protect her daughter amount to “neglect”? Nevertheless, CPS used the wording of the petition to serve its “contractual” purpose of convincing the family court not to restore custody to the mother.

A few months later, when the mother picked up her daughters for a visit, the elder girl complained of pain in her vagina so severe she

wanted to be taken to a hospital. The mother did so. This time, the medical evidence of abuse prompted officials to report that there had been “sexual offenses” against the girl, including “aggravated sexual abuse in the first degree . . . in the second degree . . . and/or rape in the second degree.”⁵⁰

Confronted with this new evidence of abuse, CPS began to change its tune: now it too accused the father of abuse, and had both children placed in foster care instead of in the father’s custody. But even then, CPS’s attack on the mother did not abate. Now the agency charged the mother with neglect, not for giving “false” information to the doctor, but for “failure to protect” her children from abuse. In other words, when CPS disbelieved the mother, she was guilty of neglect because her information alleging abuse was “false”; but now that it was considered true, she was as guilty as if she had never reported it at all! This patently baseless allegation against the mother was used, like other charges we have discussed above, to persuade a family court to place the children in foster care rather than back in their mother’s custody.⁵¹

In fact, CPS’s new attitude was only a facade. Hardly had the children been placed in foster homes when CPS dropped the charges against the father, while still maintaining that the mother was “neglectful.” Now the stage was set for the father to regain full custody. CPS bowed out of the case, having taken no action of any kind, throughout its long involvement, to protect the children from abuse.

Having been the target of one false CPS petition after another, the mother turned to activism. Despite being placed under a gag order by the court forbidding her to speak out about her case, she marched every weekday in front of the family court for a month, together with about thirty other women, carrying placards denouncing CPS and family court. She also went on a week-long hunger strike to seek an investigation of the family court system.⁵² As her protests continued, she received support from an important official, the chief medical examiner for the

county where the litigation had been taking place. He told *New York Newsday*:

This could be one of those cases where mistakes were made down the line. I have reviewed all the records.... I wrote an affidavit to the court. But I have run up against a brick wall.... There is much in this case that is suspicious, but it's been screwed up.⁵³ Yet, despite the public support for this woman and the public criticism from the chief medical examiner, the mother—who had already lost custody—lost all visitation rights with both daughters as well. After years of failed attempts to protect her children, this mother found peace in the belief that final salvation was at hand. As part of her daily routine, the mother read from Revelation those verses dealing with the Last Days. Her critiques of the family court system became almost biblical in tone: she has said she “strongly” believed that a few “token children would first have to die” as a “sacrifice” to the “evil system” before it improves.⁵⁴

The Mother Who Failed to Prevent Abuse but at the Same Time Made an Abuse Report That Was “Not Totally True”

In 1986, a mother in Brooklyn, New York, lost custody of her six-year-old daughter for allegedly failing to protect the girl from abuse that, according to CPS, never happened.

The couple had divorced in 1983, and the mother—who, by agreement, had full custody of the couple's only child—took the child to live in her parents' Brooklyn home. In May 1986, during a visit by the girl's father, the child's grandmother saw the father holding the six-year-old girl tightly on him as he lay on his back, his pants open, his pelvis gyrating underneath her. When the grandmother separated them, she saw that the father and the girl were both “wet.” The girl later confirmed that

her father had played this “game” with her several times, and that she did not like it.

The girl’s mother had not been present during the visit, but when she learned what had happened she agreed with her mother that the father should not visit the girl, or at least not without supervision. He then filed an action against her, seeking full custody of the child. The Brooklyn Society for the Prevention of Cruelty to Children (BSPCC) was the agency assigned, under state and city contract, to investigate (and pursue) child abuse cases in Brooklyn at that time. The girl confirmed her grandmother’s eyewitness account of sexual abuse to BSPCC’s investigator. But after a cursory investigation—during which the father refused to be interviewed—the agency filed a petition that charged *both* parents with child abuse or neglect.⁵⁵ The charges against the father arose simply enough from the eyewitness report of his mother-in-law. Those against the mother, however, contained an Orwellian ambiguity. She was accused either of failing to protect her daughter from abuse or of endangering the girl by reporting a “situation” that was “not totally true.” In other words, she was guilty of neglecting her daughter whether or not her ex-husband had abused her. *He* might be innocent; *she* was guilty under any circumstances. And yet no one (apart from CPS) had ever accused her of any mistreatment of the child whatsoever!

CPS’s petition (like others we have examined here) was sworn to by a person who knew nothing of the facts. That was natural, perhaps, considering that it contained critical misstatements. The mother was absent from the house when the alleged abuse occurred (and so could not have prevented it); and she did not make the report (and so could not have made a false one). Yet, with a bizarre double standard we have seen in other cases discussed previously, CPS was prepared to condemn her based solely on charges made against her ex-husband. The mother appeared for a court-ordered psychiatric examination and produced her daughter for examination as well. On October 17, 1986, before the results of any tests were available to the court, and no evidence of any kind

suggested any danger to the child while in her mother's home, a Brooklyn Family Court judge threatened the mother that if she did not undergo a second, psychological, evaluation within fourteen days, the girl would be removed from her home and placed in foster care. This threat was all the more remarkable because the judge claimed, falsely, that the mother had agreed to such an evaluation and that the father had already had a psychological evaluation. (He had not; nor was there any earlier agreement that either parent would have a psychological exam.) CPS's lawyer, who knew full well that these statements were not true, stood silent.⁵⁶

CPS's real agenda became clear at once. The mother made an appointment for a psychological evaluation the business day immediately following the court's order. But on the way to the evaluation, she suffered chest pains and checked herself into a hospital, fearing a heart attack. Doctors concluded she was suffering from anxiety—hardly to be wondered at, under the circumstances.

That was enough for CPS. The very next day, without even giving notice to the mother or her lawyer, CPS's lawyer demanded—and got—an order from the family court removing the young girl into foster care. CPS's lawyer angrily complained that the mother was "resistant" to the psychological evaluation—even though she had agreed to the appointment and had only missed it for medical reasons—and went on to say, brutally, that even if the father *had* sexually abused the girl ("What he did or didn't do to this child, that may or may not amount to sexual abuse, I do not know"), her daughter should be removed forthwith from her mother's home, because "if she [the mother] has anxiety attacks, then maybe she can't care for the child, and maybe the child is suffering the same anxiety attacks she is suffering."⁵⁷ He also accused the mother, falsely, of refusing to obey court orders.⁵⁸ (She had not violated any court order.)

As a result of CPS's misstatements, Brooklyn Family Court removed the girl from her mother's home. By contrast, the father, who had been

accused of abuse by an eyewitness (and the child victim), had refused to be interviewed by CPS, and had not been seen by a psychologist, got red-carpet treatment from CPS, which would soon drop the charge against him altogether.

It is worth noting that the removal caused by CPS was not only unnecessary and cruel—it was illegal. First, it was made without a constitutionally required hearing. Second, the removal violated New York law, since there was no showing of overriding necessity.⁵⁹ Finally, the removal violated the family court's own order, which had granted the mother fourteen days to have the psychological evaluation, of which only four had elapsed when the judge signed the removal order.⁶⁰

CPS had not finished with its attacks on this mother. When the mother's lawyer sought the dismissal of the charges against her, based on the obvious factual errors in the petition, CPS simply rewrote the petition to allege that the mother had caused the girl to develop symptoms of abuse by wrongly believing the abuse had occurred. When it was pointed out that no expert who had evaluated the child, or the mother, had ever made such a claim, CPS and the family court judge quickly located another expert—the notorious Dr. Arthur Green—who (after talking to the judge and being told that no abuse had occurred) obligingly diagnosed the mother as suffering from a “circumscribed psychosis” that enabled her to transmit symptoms of abuse to the child. (This sort of bizarre mental health diagnosis, seldom encountered anywhere but in family courts, will be discussed in Chapter 7.) CPS's lawyer also told an evaluating psychologist (as he later admitted during trial) that the mother was “paranoid” because she believed her daughter was going to be put up for adoption while in foster care. Yet this fear could hardly be called “paranoid”—it was, in fact, completely justified: an official of the foster care agency itself admitted in testimony that he had told the mother her daughter would be adopted.⁶¹ To CPS, anything the mother said or believed, even when it was true, was proof of “mental illness.” As in other cases we have discussed, once CPS had secured the child's removal into

foster care, it abandoned the charge against the father. It also abandoned the factual claims made against the mother and amended the petition no fewer than three times in order to ensure that the mother would never regain custody.

The plan worked. The mother never regained custody; in fact, family court ultimately awarded the allegedly abusive father full custody of the child. Almost at once after going to live with him, the girl (then eight years old) declined into anorexia so severe that her life was threatened; a treating physician called her “by far the worst case of emaciation I have ever seen.”⁶² The mother rushed her daughter to a hospital for emergency treatment (the father, a physician, had refused either to hospitalize her or to monitor her condition with necessary blood tests), an act which, according to later testimony, may have saved the girl’s life.⁶³ How did CPS respond? It refused to investigate the father either for continuing sexual abuse (severe anorexia in a girl so young is often a symptom of sexual abuse) or for medical neglect (for failing to treat his daughter’s life-threatening condition). Family court then condemned the mother for giving the child medical treatment without the father’s consent and cut off the mother’s visits and telephone contact, even barring her from writing the child letters.

This mother turned to activism, appearing on dozens of television and radio programs to publicize what had happened in her case and others. On several occasions, when CPS learned of an upcoming appearance, CPS’s lawyer contacted the show’s producers and maligned the mother to them.

This mother, like some of the others whose stories have been told here, bore long-lasting scars from the senseless destruction of her relationship with her daughter. For years, she did not buy new clothes or date men. In fact, she did not even go for gynecological examinations. She told family members that she could not think of herself as a woman, let alone as a mother. And the thought of having another child was terrifying: what if this one, too, should be taken away?

CONCLUSION

The case histories summarized in this chapter are representative of a national problem. When sexual abuse allegations are involved, CPS agencies make crucial decisions about the fate of children without any apparent concern for the children's interests. Accusations seem to be made, instead, with another agenda in mind: the desire to remove a child from a protective parent, to place the child in foster care, and then to take whatever final steps are necessary to make it appear that no abuse could ever have occurred. If this means basing neglect petitions on rumors, baseless tips, unfounded "diagnoses," or even outright untruths, CPS will take that step. Unchecked power and unqualified secrecy give CPS agencies the power to do virtually whatever they please.

In recent years, the child welfare system has come under attack for letting children die or suffer serious maltreatment in their homes. CPS's defenders have replied that the agencies are understaffed and overworked. The eagerness of the system to remove healthy children into foster care for the ultimate purpose of placing them in the custody of men credibly accused of sexual abuse is, therefore, particularly outrageous in the context of a CPS system supposedly too "overburdened" to protect children from real abuse.

For instance, from *Child Maltreatment 2002*, the annual report prepared by the National Child Abuse and Neglect Data System (NCANDS),⁶⁴ we learn that "approximately 1,400 children died of abuse or neglect during the year 2002, a rate of 1.9 children per 100,000 children in the population."⁶⁵ Perhaps the time that CPS spends "rehabilitating" relationships between sexually abusive fathers and their child victims could be better spent preventing child fatalities.

All in all, it should be clear that the madness of today's family courts would be impossible without the assistance of CPS agencies, which use precious resources to pursue false charges against innocent mothers who have reported suspicions of sexual abuse.

ADDENDUM—ESCALATION OF ASSAULT ON MOTHERS AND IDENTIFIABLE RACIAL BIASES AGAINST BIPOC MOTHERS

Spurious Charges against Protective Mothers—Conflating “Abuse” and “Neglect”

Shortly after the publication of *From Madness to Mutiny*, we were invited to write a commentary for the *Journal of Child Sexual Abuse* on how Child Protective Services (CPS) responds to protective mothers' complaints of child sexual abuse. We pointed out that mothers were being prosecuted by CPS in “[t]he absence of any logical criteria behind the charges of neglect” being brought against them by CPS, even in the face of credible evidence of abuse committed by the other parent.⁶⁶ Several years later, the US Department of Justice, Office of Justice Programs (an agency of the US Department of Justice focusing on crime prevention), would post our commentary piece on their website.⁶⁷ This egregious practice on the part of CPS in bringing patently false charges against protective mothers goes unchecked because the appellate courts have been woefully neglectful in applying the law. As such, they have either thoughtlessly rubber-stamped these spurious charges by affirming the lower court's decision against the mother or they have refused to hear the mother's appeal altogether, deeming the mother an “unaggrieved” party (one who has no grievance to warrant an appellate review).⁶⁸

In researching more recent cases, it has become apparent that the CPS backlash against protective mothers has intensified, leading to further distortions of child welfare law and encouraging more wrongful state interventions into the privacy of family life. What has been observed is that whereas in prior years it was common to charge a protective mother with “neglect” in response to her complaint of sexual abuse (even when supported by credible evidence), today CPS is freely charging protective mothers with the more serious charge of “abuse”—in lieu

of a neglect charge—without even a scintilla of evidence to warrant an “abuse” charge, nor a “neglect” charge either. To draw an analogy to a criminal context, this would be similar to bringing the serious charge of “capital murder” against an unsuspecting person, just as a form of harassment, instead of the often-used misdemeanor charge of “disorderly conduct”—customarily used to gain control over a person in a difficult situation. Thus, the actions of CPS in bringing “abuse” petitions against protective mothers, in place of their formerly used “neglect” petitions, is a clear indication of how they have intensified their assault on protective mothers in recent years.

How Child Protective Services Racially Discriminates against BIPOC Mothers

In analyzing the more recent protective mother cases, one must remain cognizant of the racial factors that come into play. These factors have naturally become more salient as the mere number of Black, Indigenous, and People of Color (BIPOC) protective mothers has increased overall. This is not surprising given the fact the BIPOC population in general has grown appreciably over the past twenty years, contributing more than any other group to population growth in the United States.⁶⁹ As a result, BIPOC mothers now constitute a larger percentage of protective mothers as the BIPOC population itself has increased overall. Thus, they have become increasingly more visible among protective mothers. Consequently, close attention must be paid to how the family courts and CPS treat people of color. Our analysis of CPS cases points to the fact that racial prejudices shown by those working either full time or as regular consultants for child protective service agencies (e.g., caseworkers, psychologists, and medical doctors) are a common occurrence and should not be ignored.

Specifically, one recurring pattern of such prejudices is the insertion of gratuitous statements in CPS documents (e.g., petitions, child safety plans, and case summaries) about mental health “pathologies”

(e.g., pointing to psychosis) in BIPOC protective mothers, and possibly their children, when there is no basis for such diagnoses in the first place. This follows the general prejudices across the mental health profession in which people of color are more frequently diagnosed with psychosis whether it is warranted or not.⁷⁰

Moreover, another recurring pattern, as seen in the documents contained in CPS files, is to ridicule the BIPOC mother's mundane behavior, albeit without warrant (e.g., as "bizarre," "peculiar," or "out of control"), even if it doesn't rise to a level of psychological abnormality. Interestingly enough, notwithstanding the lacing of the CPS files of BIPOC mothers with pejorative labels and mischaracterizations, the case outcome for BIPOC mothers and their Caucasian counterparts are basically equivalent: that is, both are equally dismal. Comparative Analysis of BIPOC with Caucasian Protective Mothers We present two case histories where we analyze the role of CPS in each case. Controlling for as many variables as possible, we chose two concurrent cases with geographic propinquity. That is, both occurred around the same time, and both were located in Cook County, Illinois. In addition, both cases were of the same socioeconomic status and both had high-priced counsel representing the mother. The cases were similar in other ways, too. Each involved a man credibly accused of sexually abusing his daughter. And in each case, after the mother took her daughter to a local hospital for suspected sexual abuse trauma, it was medical staff at the hospital — that is, mandated reporters — who reported suspicions to CPS that the father had sexually abused the girl. Thus, neither mother reported abuse themselves.

Given that family court cases are confidential and thus closed to the public, and that there is also no repository or database from which one can simply pull these cases, we did extensive research to find these two cases for comparative analysis. As a condition precedent we required a close match of demographics, medical history, and litigation trajectory.

What we found is that while these two cases closely mirror one another, their narratives begin to diverge considerably when factoring in race.

Specifically, whereas both mothers were accused by CPS of fabricating sexual abuse claims, the local CPS agency went further in its condemnation of the BIPOC mother, whom its caseworkers accused of causing her three-year-old daughter to experience “hallucinations”—even though there was no medical basis for this charge. Although hallucinatory thinking is found in psychotic illnesses, it is not surprising to see how carelessly such terms may be tossed around when describing minority families. That is because BIPOC mothers, similar to the general minority population, as mentioned above, have been freely ascribed diagnostic labels strongly associated with psychotic illnesses, even in the absence of an underlying condition that would point to a psychosis or similarly to any symptoms that might be associated with any form of psychosis.

Such racist views no doubt add another layer of injury for a BIPOC mother struggling to protect her child from sexual abuse since it stirs up memories of previous racially motivated attacks on one’s character and integrity. In analyzing these two cases that evince such similar fact patterns, we were able to see with unusual acuity how racism played a significant role in how the local CPS agency responded to each of these protective mothers, respectively. We begin our comparative analysis with the Caucasian protective mother so as to show in the latter case how racial prejudice further exacerbates an already noxious situation, thereby adding another facet to the protective mother debacle.

Cook County Caucasian Mother Seeks Medical Intervention for Child

The Cook County Caucasian mother brought her six-year-old daughter to the emergency room for rectal bleeding following an overnight visit with the father. Returning from the visit, the child had made alarming disclosures to the mother about her father coming into her bedroom at

night and forcing himself on her. She reported both sodomy and rape to her mother. At the hospital the child was interviewed by an emergency room nurse in the absence of the mother. The child disclosed to the nurse “that when she goes by her dad’s house while she is sleeping he puts his penis in her vagina and in her rectum.” The child then clarified to the nurse that during these nocturnal attacks she would “just pretend like ‘I’m sleeping.’” The nurse stated in her report that she “asked her what it felt like when he does this to her and she [the child] said ‘it’s hard and hurts me.’”⁷¹

The nurse recommended that the child be taken via medical transport to another area hospital that specialized in child sexual abuse trauma. At the second hospital, a full diagnostic workup for child sexual abuse was performed. The hospital report concluded “child sexual abuse, confirmed.”⁷² The report contained a set of medical forensic documentation forms with explicit diagrammatic markings of the physical trauma to the child: (1) “right mon pubis with 1cm oval bruise with surrounding erythematous papules”; and (2) “erythema” of the genital area.⁷³ The hospital immediately called DCFS (Department of Children and Family Services), the local CPS agency in Chicago, to make a report of child sexual abuse against the father. However, DCFS made a finding of “unfounded.” This was despite medical evidence and the child’s own report of repeated incidents of alleged sodomy and rape by the father while she was in bed at night.

The mother went back to family court and pleaded for protection. The judge ordered “supervised” visitation between the child and the father at the father’s home. This, itself, was ostensibly a canard, in that the father was instructed to have the nanny sleep in the room with the child when she would go to her father for the weekend. When the nanny was deposed by the mother’s counsel, it was discovered, however, that her sleeping arrangements had left the child at times unattended.⁷⁴ Over the next year the child was allegedly subjected to some additional incidents of sexual assault on weekends while with the father. When the mother

brought her daughter into the hospital for one of those subsequent incidents, the hospital found an “anal fissure,”⁷⁵ which was reported to DCFS.

Child Protective Services Uses Foster Care to “Cover Itself” In response to the hospital’s report of an anal fissure, which could be consistent with sexual abuse, DCFS asked the court—in Illinois, the court that handles CPS petitions is called “Juvenile Court”—to have the child removed from the mother to be placed in kinship foster care with a relative of the father’s family. This quick move by CPS to take a child away from its mother immediately after a suspected sexual assault by the father has become all too common in recent years, and it is now happening even more rapidly and more often in protective mother cases. Such legally unjustified assaults on parental rights should not be taken lightly. After all, placing a child in foster care is insult on top of injury: for a child already traumatized by sexual assault to then lose its cocoon of safety is contraindicated at best, and unconscionably poor practice at worst. Such stratagems, albeit traumatic to the child and certainly a flagrant misapplication of child services laws, nevertheless allow CPS to “play it safe.” That is, they remove the child from the mother just in case the mother “lied” about the abuse, but since they are not sure whether the abuse happened or not, they place the child in foster care so that they’re not accused of exposing the child to harm. What makes CPS’s action in this case unconscionable is that the child already had a home: a warm, loving environment living with their mother. So, why remove the child to foster care? In fact, while in foster care it appears CPS, according to the files we reviewed, did not make extensive efforts to review medical reports, interview credible witnesses, investigate the father for sexual abuse, or evaluate the child for sexual abuse trauma to determine the safety of placing the child with the father after foster care. Similarly, they did not appear to be investigating the mother to determine her fitness so that the child could be returned to her. So, the removal of this child to foster care is perplexing. Unfortunately, current cases point to

an escalation in CPS asking courts to abruptly remove children from mothers to be placed in foster care following a mother's attempt to seek medical treatment after an incident of sexual assault committed by the father. Hence, such duplicitous placements of sexually abused children in foster care have become more or less the standard operating procedure in protective mother cases today. But there is another factor driving these indefensible foster care placements in response to a mother's good faith attempt to protect her child.

And that factor is sinister. In case after case, we have witnessed how CPS uses foster care as a convenient segue toward transferring custody of the child to the father accused of sexual abuse of his child. In truth, foster care placement works very well with this sinister plot to shovel abused children into the hands of the parent they have accused of abuse. Namely, because it may be hard to justify taking a child away from a mother without a preponderance of evidence to show maternal incompetence, if CPS first removes the child from its mother (albeit on spurious grounds), it makes it that much easier to transfer custody over to the father. In other words, if the auxiliaries, such as CPS caseworkers (and law guardians), were not present in family law litigation, the father would have the burden of presenting witnesses to prove the mother of his child is incompetent and that he is the better the parent. But once CPS swipes his child from the mother, using foster care placement as the segue to a custody transfer over to him, then the father is essentially "off the hook" with having to put on a case for himself. These reckless assaults on mothers' rights by CPS agencies — funded with taxpayer money — reveal a rotten streak at the core of the system, an appropriate subject for a federal investigation that might uncover the motive behind such behavior.⁷⁶

Child Protective Services Transfers Child to the Father—with Restricted Supervised Visitation for the Mother

Not surprisingly, after the child had been in foster care for three months, the law guardian and the CPS lawyer asked the Juvenile Court to place the child in the father's custody. The court obliged and gave the mother limited supervised visitation with her daughter. The court at the same time forbade the mother from taking her child to her home, even though a supervisor would be present. Without providing any substantial reason, the court's order forced the mother, who had a comfortable home in a bucolic section of Chicago, a scenic wooded area, to wander in public places for her visits with her daughter. In nice weather this would not be a problem, but in the windy city of Chicago where frequent subzero temperatures can be brutal, the mother had to go inside a public building to visit with her child.

The conditions of these visits had an effect on the child over time. Certainly, the traumatic rupture of this child from her mother compounded by "visitation vagrancy"—where the mother and child had to meander in public places for over three hours for each visit—left the child unable to seek the comfort of her familiar surroundings: her bedroom, her toys, the family room, the kitchen, and any place where warm memories have been made. Visitation vagrancy, as intended by CPS and the Juvenile Court, had ominously proven to be a "success." Three years of hunting down public places for visitation with her child—monitored at all times by a supervisor listening into the child's conversations with the mother—left the child feeling listless about her visits with her mother altogether. This, in fact, is not an uncommon reaction on the part of children since such highly abnormal visitation conditions are not conducive to promoting the child's sense of security and comfort. In fact, on Christmas Day, when most stores and restaurants were closed, the mother had to find a tree to sit under with her child, as she couldn't bring her daughter to her home to sit on a couch opposite the living room

fireplace. Instead, the mother had her visit with her precious daughter on the cold ground; she sang Christmas carols to her daughter while opening up the gifts under a bare tree. Both mother and child were so cold that they soon went back to the car—crammed with mother, child, and supervisor—to finish their visit.⁷⁷

The financial toll on this Chicago mother for such visitation arrangements must be noted. The mother, who worked as a waitress, was ordered by the court to pay the visitation supervisor \$315 per week (for seven hours split between two visits per week). However, as a precondition to being allowed to have such monitored visits with her child, the mother was ordered to undergo weekly “therapy” (costing \$150 per session) to cure her of her “misperceptions” about the father’s abuse. When compared to other cases, this mother had gotten off relatively “easy,” in that most mothers must pay, in addition to visitation supervisors, the costs for a cold, office-like visitation setting to host their visits with their children—usually at a facility that contracts with the courts.

CPS Contorts the Law—Inventing New Ways of Prosecuting Protective Mothers

This mother, after losing custody to the father in 2019, would spend the next three years defending herself against a fierce and unrelenting “prosecution” by the DCFS in the Juvenile Court on the charge of “medical child abuse”—a mere invention of CPS agencies to punish protective mothers for seeking medical care for their sexually abused children. The language of this charge deserves some comparative analysis. In prior years, when mothers had been faulted by CPS for seeking medical treatment for their sexually abused children, they were charged by CPS with “neglect” and not “abuse.” They were accused of having a psychiatric condition, such as Munchausen Syndrome by Proxy (also referred to as “factitious disorder by proxy”), propelling their relentless search for medical treatment for their sexually abused children, treatment which

CPS, in denying the abuse, claimed was not needed. As a result of this purported mental condition, mothers had been charged with “neglect,” and not “abuse,” as the latter, both statutorily and according to case law, is a more serious charge since it engenders a greater risk of termination of one’s parental rights. Although parental rights can be terminated for neglect as well, CPS must go through another step by proving *permanent neglect* as a necessary precondition for severing a mother’s parental rights. In contrast, a finding of abuse is much more likely to stand on its own, without any additional proof of “permanence” in order to sever a parent’s rights to their biological child.

In this case however, the mother was not even accused of Munchausen’s Syndrome by Proxy or any permutation thereof; nor was there an iota of evidence that she repeatedly questioned her child about abuse or that she sought medical services that were unnecessary. On the contrary, each time she brought her child in for care the medical facility called in a report of abuse or transferred the child to a higher-level facility that could make that determination, which they affirmatively did. Yet, DCFS steamrolled ahead with an “abuse” charge against this innocent mother. As CPS has become more emboldened over the years, the charges they bring against protective mothers have commensurately been more serious. As this case instantiates, what we see in protective mother cases today is that “abuse,” rather the weaker charge of “neglect,” is often leveled against the mother and, ironically, without even needing to invoke a mental illness diagnosis (whether genuine or not) to justify such a serious charge in the first place.

In an attempt to formulate the “medical child abuse” charge brought against this Chicago mother, DCFS relied on a physician to review the file, whose diagnostic opinion was given much weight even though her role admittedly was not to interview the child. Nonetheless, the expert summed up: “in my opinion . . . this child is suffering from a variation of medical child abuse, that is when a parent fabricates symptoms that lead to medical treatment that is [in] fact unnecessary” (emphasis

supplied).⁷⁸ The key word here is “variation” insofar as the medical expert formulating this charge of abuse appeared to be cognizant of the fact that charging this mother with “medical child abuse” deviated from the standard definition of medical child abuse. At the core of the expert’s opinion that the child had been “coached” by the mother, is that the child had been “explicit” in describing her father’s anatomy. Nevertheless, this same expert cautioned “that the father and [the child] bathing together at this age [the child was seven years old] is inappropriate and should stop since [the child] has now started to notice and ask about genitals.”⁷⁹ Given the undisputed history of the father taking off his clothes and immersing himself in the bathtub with his daughter who is now, according to this expert, old enough to be aware of her father’s genitalia, why would it be surprising that the child would be anatomically explicit in describing her father’s alleged sexual acts, as opposed to having been coached by the mother to do so?

In the absence of solid evidence that the mother had been planting ideas in the child’s mind, the expert chose to speculate. But she did so with an air of diagnostic certainty that one only sees in the contorted world of family courts; in all other settings physicians generally maintain a degree of uncertainty as integral to the canonical paradigm of “differential diagnosis,” which has proven essential for proper medical decision-making. In the summary section of her report the expert stated, “I am sure the mother is talking to [the child] about this [sexabuse] frequently as the mother has an anxiety disorder.”⁸⁰ In fact, the expert acknowledged the mother was following her treatment plan of medication and talk therapy for her condition of generalized anxiety disorder, which not surprisingly occurred only after the mother was subjected to the trauma of losing her young daughter through the courts. All in all, the mother’s symptoms were now under control; with an amelioration of her condition, the mother was not likely to be overreacting.

The expert concluded that “[the child] is being sexually abused by [the] mother due to sexually inappropriate discussions”⁸¹ —certainly a

bizarre way of defining “sexual abuse” that some judges have injected into their courtroom rants against protective mothers, although not often seen in medical reports evaluating child sexual abuse trauma. Or perhaps the madness of family court judges has finally spilled over into the medical profession portending more of these kinds of novel interpretations of sexual abuse aimed at condemning the innocent mother and vindicating the allegedly guilty father?

Actually, one must struggle to find any rationality to the charge of “medical child abuse” that would warrant the state of Illinois intervening in the privacy of family life. Although the expert formulated a diagnosis of “medical child abuse,” she conceded that no one had yet assessed the child for “emotional impairment.”⁸² She based her diagnosis of medical child abuse on the mother’s bringing the child in for medical evaluations which the expert found unnecessary, even though the mother was clearly responding to incidents of rectal bleeding when the child would come home from seeing her father, which were supported by the disturbing disclosures made by the child about what took place during those overnight visits. All in all, in the absence of any findings of emotional impairment of the child one has to wonder where the evidence of “child abuse” that has occupied over three years of CPS courtroom litigation and its attendant paperwork—safety plans, assessments, and reports—has been tucked away. In fact, as seen in the Adjudication Order of the Juvenile Court two years after this medical expert’s report, there was still no evidence of any “emotional impairment” to the child.⁸³

However, inasmuch as CPS concocted a new definition of “medical child abuse” the Juvenile Court likewise applied a new standard for judging the mother by stating that “impairment was *not* needed” (emphasis supplied)⁸⁴ to find the mother guilty of this bizarre charge. Instead, the Juvenile Court determined that all that was needed to make a finding of child abuse or neglect⁸⁵ against this mother was an opinion from an expert that the mother’s actions of taking the child for medical evaluations “has negative impact [*sic*] on [the child].”⁸⁶ One has to re-

alize that if the state were to intervene in the lives of Americans every time someone did something that purportedly had a “negative impact” on another, we would be living undeniably in a dictatorship. In fact, the mother’s attorney correctly pointed out to the judge at a recent Zoom hearing that CPS’s hijacking of this case from the domestic relations court—where divorce, custody, and financial support matters were being heard—has led to a colossal waste of taxpayer dollars, especially that this CPS canard had been going on in Juvenile Court for already three years.⁸⁷

Sadly, this case is not an anomaly. When reading the hospital reports or, even, the police reports that stated that the child reported to them that her father had instructed her “if her mom brings her anywhere to tell everyone it’s a lie,”⁸⁸ the case appears straightforward and simple. That is, the doctors at the hospital found abuse, and the child told the police her father instructed her to lie to everyone by denying that she had been abused by the father. However, reality starts to unravel and fall completely apart when reading the CPS file, wherein a wholly different picture is presented. That picture showed that the CPS medical expert, who formulated the “medical child abuse” charge, had not interviewed the child, nor had CPS provided for her first-hand review the medical reports from the hospitals that called in child abuse reports against the father based on their medical findings and interviews with the child that were conducted outside of the presence of the mother.

Tragically, the lives of children are at stake as child protection twists and contorts statutory and case law definitions, making up its own rules to support the father accused of sexual abuse while the mother can be prosecuted and impoverished as she struggles to free herself of the oppressive and forceful powers of CPS. In fact, this Chicago mother, like far too many protective mothers pursued by CPS, saw that they have to wait years just to finalize their divorce. This is so, because in most states once the case gets into Juvenile Court or Family Court, the divorce proceedings are generally stayed pending the resolution of CPS’s

case against the mother. And although the laws in most states favor final case dispositions in a reasonable amount of time (approximately twelve to eighteen months), these spurious, “custom-made” child protection proceedings used against protective mothers tend to last much longer. Without editorializing, the facts show how mothers, unable to finalize their divorce while trapped in these contorted CPS proceedings, find themselves shackled to their abusive husbands by the madness of the family court and child welfare system—where they are deprived of a fundamental constitutional right to raise, care for, and educate their children as protected by the Due Process Clause of the Fourteenth Amendment to the US Constitution.

Mother Threatened with Jail if they Shared their Torment by the Courts with Others

This Chicago mother was threatened by the judge with contempt of court (she would be jailed and fined) if she were to share with anyone what transpired in her court proceeding.⁸⁹ This included her mother, her siblings, her support group members, and her fellow churchgoers. Similarly, just a year earlier, a New Jersey high school science teacher, who was prosecuted for “neglect” (based on “brainwashing”) after her children’s pediatrician made a report of suspected child abuse against the children’s father, was warned by the judge that she would be held in contempt of court if she were to speak even casually to her parents, with whom she lived, about the family court proceedings. In both the Chicago case and the New Jersey case, and in so many other protective mother cases, forbidding discussion with family members and friends was illogical since they were not going to be called as witnesses in the proceeding. So, there does not appear to have been any sound legal reason to forbid either this Chicago mother or the New Jersey mother, or any of the other protective mothers, from speaking to her family mem-

bers and friends about her painful ordeal in the family courts and child welfare system.

By isolating the mother from her primary support system, the trauma the mother endures is compounded. In fact, as previously discussed, most mothers have been forced into court-ordered therapy with experts who rationalize the system and challenge the veracity of the mother's perception of the madness suffusing the courts. In fact, this "gaslighting" of mothers is a very common theme in the American family courts, which many critics have likened to a "modern-day gulag." One of the authors of this book (Amy Neustein) wrote, in a lengthy memorandum to then-United States Attorney General Merrick Garland (under the Biden Administration), about CPS's "gaslighting" of mothers so as to obliterate the truth of what happened to their children:

The cover-up of sex abuse ... in which CPS goes on the offensive against the mother, using every opportunity to attack the mother's credibility by impugning her sanity, instantiates a national pattern of "gaslighting" of mothers. Such harassment by CPS is not *without* injury to the mother because, in addition to the loss of their children as a result of these all-too-common conspiratorial cover ups of sex abuse, mothers can begin to doubt their *own* reality so as to conform to the *false* reality of an agency that is working very closely with the father to suppress sexual abuse crimes committed against the children. Such efforts to manipulate a mother's mind, especially when her children have been taken from her by the state without a constitutionally valid hearing, demonstrates extortion via an extreme form of *harassment* (viz., gaslighting). To wit, the CPS officials are "playing with" the mind of the mother in order to force a recantation from her so that the father will be exonerated. Using government grants and other means of public funding in order to oppress, subjugate, and otherwise overpower a mother by forcing her into "therapy" with the CPS

[approved] psychologist for the sole purpose of washing away a record of abuse committed by the father enables taxpayers to bankroll this criminal conspiracy.⁹⁰

BIPOC Mother Suffers Ordeal with Child Protective Services—and Fears her Own Hired Counsel

In comparing the BIPOC Chicago protective mother case with that of the Caucasian Chicago mother, whose case was just presented in detail, one finds the additional factor of “mockery” of the BIPOC mother, as evidenced in the CPS reports wherein purely gratuitous comments are made about the mother to serve the purpose of denigrating her and stripping away her integrity as a mother. But before that is discussed, we will give a brief history of what transpired in this case.

On Christmas Day 2020, the BIPOC mother brought her three-year-old daughter to the emergency room at a major medical center in Chicago following a visit with the father. The resident who examined the child notated in his diagnostic workup that that the child had “bruises noted on the back/flank and buttocks.”⁹¹

The young child repeatedly told several members of the staff: “my vagina burns, Daddy kissed it.” She then went on to speak of the physical assault she endured that resulted in the bruises on her back, her side, and on her buttocks: “He [her father] whooped me.” The physical injuries and the child’s disclosures were a red flag for the staff at the hospital. As a result, in the Assessment/Plan section of the hospital report it stated: “[V]erbal comments concerning for sexual abuse . . . RN notifying DCFS + police [*sic*].”⁹² The mother, however, according to the hospital expressed great fear of retaliation from the father, and reproach from her own lawyer for bringing her child to the hospital.

Although the child had a set of bruises covering a relatively large portion of her body (her back, side, and buttocks) and was complaining that

her “vagina is burning [sic]” and that her father had placed his lips on her private parts, the mother, as just mentioned, feared retribution from the father, as well as a scolding from her own attorney, for seeking medical intervention under these trying circumstances. In fact, the nurse notated that “about 40 minutes was [sic] spent by this RN explaining to mom regarding consent for medical forensic exam and evidence collection, photograph evidence, and reporting decision evidence analysis [sic].” She went on to explain: “Mother reports feeling fearful of retaliation from father if he finds out and having her lawyers upset with her for doing this.”⁹³

Finally, after these forty minutes of the nurse encouraging the mother to allow a vaginal exam of her child and the collection of specimens for a possible STD, and the reporting of the case to DCFS, the mother finally conceded to allow the hospital to conduct their requisite forensic workup of the child.

The angst this mother showed in the face of such disturbing evidence—and in particular her fear of her own attorney—is commonly found in the BIPOC population of protective mothers since they are often met with derision from the child protection system and from their own counsel when they avail themselves of the healthcare services supposedly accessible to all members of the population, although apparently not to BIPOC mothers who carry the extra burden of suffering from racial discrimination. In contrast, in the previously discussed Caucasian protective mother case, the Chicago mother had been on the phone with her attorney for several hours as he calmly reassured her while she was in the emergency room with her daughter after an alleged sexual assault incident when visiting the father.⁹⁴

Both the Caucasian and the BIPOC mother had the same socioeconomic status; both had hired high-priced attorneys in the downtown Chicago area and were being charged the same hourly rate. But while one received crucial moral support from her attorney, the other feared condemnation or worse. This speaks to the added burden placed on the

shoulders of BIPOC mothers who encounter racial prejudice and hostility even from those to whom they turn for help. The BIPOC mother's fears were proven to be well founded. Similar to the Caucasian mother, the BIPOC mother was charged by DCFS with "medical child abuse" for trying to seek proper medical intervention for her child. But the comments of the medical expert who had been similarly hired by DCFS in the othercase, in which she likewise made a diagnostic finding against the mother for medical child abuse, took on a particularly contemptuous tone in her report on this BIPOC mother.

BIPOC Mother Mocked by Child Protective Services—while Evidence of Child Abuse is Ignored

For example, the medical expert hired by DCFS started off her lengthy report by stating that the mother "has no control over [the child's] behavior" which she based on the finding that the child, who was barely four at the time, "threw temper tantrums" and "was excessively actively [*sic*]." ⁹⁵ Temper tantrums in children age two to five are very common and certainly not a reflection on the mother's parenting skills. The child's IQ was extremely high, and the mother knew she was raising a gifted child. To engage the child's mind, she cultivated whatever interests the child had. This little girl loved animals, and the mother often took her child to the zoo. The DCFS medical expert, however, rendered a value judgment, stating the BIPOC mother had "very peculiar motivation [*sic*]" in buying her child toy snakes and "taking her to see animals to encourage her to become a veterinarian." ⁹⁶

What is noteworthy is that this medical expert made no mention of the disturbing pattern of bruises on the child's body, and the child's statements about the beatings she endured while visiting her father. Nor was any consideration given to the child's night terrors and enuresis following visits with the father. Instead, the evaluation focused on disparaging the mother for any efforts to help her child, especially in

a system where she struggled to be believed. For example, the expert asserted that the mother “demonstrated lack of boundaries” because she sent “letters via MyChart giving permission to the medical team to talk with lawyers.”⁹⁷ The mother was now undoubtedly in a compromised position. Constantly challenged by the DCFS which didn’t believe her despite the hospital reports that clearly showed concern for sexual abuse trauma to the child, she gave permission for the medical team working with DCFS to talk with her attorney so as to prove to CPS that her concerns were indeed well founded.

In truth, the right of a mother to have an attorney represent her interests and speak, when necessary, to the medical team working for DCFS in an attempt to make sure the child is protected from abuse is obviously not afforded to a BIPOC woman, even if she can afford to pay her counsel fees. Ironically, by exercising her rights to use her counsel as she sees fit, this BIPOC mother was deemed to have a “lack of boundaries.” Perhaps the boundaries that she did lack were her striving for preservation of her legal rights, her dignity, and the welfare of her child which were apparently outside of the “boundaries” set for her by a system saturated with racist views.

While forming a diagnosis of “medical child abuse” on the grounds that the child “is receiving unnecessary medical treatment [for sexual abuse trauma] provoked by the parent,”⁹⁸ the medical expert commented that the child, who recently turned four, “may need separation from her mother to allow normalizing her environment [*sic*].” The expert justified her recommendation that this BIPOC protective mother be charged by the state child protective services with medical child abuse because “[the] mother has a perception that her child is being abused and therefore [*is*] not an objective caretaker.” Sadly, the expert’s comment that the child “may need separation from her mother” eerily suggests that the child may risk being placed in foster care given the ominous conclusion of this expert: “Our recommendation to the team was to support a report of Medical Child Abuse in this case to DCFS.”⁹⁹

In fact, the CPS's removal of children from their mothers to be placed in foster care is typically more prevalent in the BIPOC population¹⁰⁰ since these mothers are caught in the stereotypes caseworkers have about purportedly substandard conditions in BIPOC homes. Here, ironically, removal from the concerned mother is mentioned as a possible consideration by the medical expert performing a consult for DCFS because the mother, according to CPS, purportedly lacks "objectivity" about the father, when in fact she is reacting with natural concern about her child's bruises, disclosures of abuse, traumatic behavior (night terrors), and regressed behavior (enuresis).

BIPOC Mother Reproached by Child Protective Services' Expert—With Added Hints of Psychopathology

As will be shown, the medical expert consultant for CPS—in using mere conjecture to draw a conclusion that this BIPOC mother was pestering her child with queries about sexual abuse—denounced the mother, using somewhat harsher language than when rendering the same "medical child abuse" finding against the Caucasian mother previously discussed. Certainly, in the Caucasian case, the medical expert reached diagnostic conclusions premised on flawed reasoning. For example, she used the mother's anxiety disorder—a condition that was acknowledged to be under control—to imply that given her condition it would be impossible for her to refrain from "talking" to her child frequently about sex abuse.

The expert's unwarranted reasoning was similarly evident in her professional assessment of the BIPOC mother. However, when comparing her two assessments, it is clear that the expert was remarkably less restrained in describing the BIPOC mother when compared with the Caucasian mother, even invoking bizarre psychopathological labels to describe the BIPOC mother.

For example, pointing to the BIPOC mother's condition of "baseline anxiety" (a general anxiety disorder found across a fairly large segment of the American population) and her purported "obsession with CSA [child sexual abuse]" the expert commented: "I am concerned that the mother is fixated on the sexual abuse, examining her daughter, asking her daughter direct questions about her vagina, symptoms [sic] and I would say I would not be surprised if the mother directly asks [the child], 'did daddy touch you?'"¹⁰¹ Notice how the expert freely ascribed to this BIPOC mother terms, such as "fixations" and "obsessions," which are generally found in patients suffering from serious forms of psychopathology—and when such terms are used in a medical context, as opposed to casual conversation, they bear clinical significance.

Although the psychiatric "labeling" of BIPOC mothers is particularly problematic, the outcome for both Chicago mothers described here was the same: they were both charged with "medical child abuse"—a confabulation that serves the needs of a CPS and court system intent on punishing a mother for making an allegation of child sexual abuse against the child's father. Nevertheless, the excoriation of the BIPOC mother would naturally leave her demoralized, exhausted, and dejected since she suffered racism as an added component to the torment brought on by malicious child protective service proceedings.